



Sexual Safety in Mental Health (Mental Health) Policy and Procedure

- ALERT**
- It is illegal for a staff member to engage in sexual activity with a consumer. Refer to the WA Criminal Code.
 - Types of behaviour that breach and/or compromise the sexual safety of a mental health patient include:
 - * Sexual assault or harassment.
 - * Consensual sexual activity in an inappropriate context/setting.
 - * Sexually disinhibited behaviour.

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Background

- Consumers accessing mental health services have a right to feel and be safe. It is crucial for mental health services to avoid traumatising, re-traumatising or compounding previous trauma and to foster a culture where consumers feel and are sexually safe.
- Mental health patients are exposed to a number of potential risks, particularly when they are acutely unwell. Often these risks are related to their own behaviour or the behaviour of other patients or are a direct result of their mental illness.
- The purpose of this policy and procedure is to support the mental health service in meeting its responsibility in relation to maintaining the sexual safety of mental health patients and promoting sexual safety to mental health staff, patients and their families/carers.

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Terminology

Sexual safety	Refers to being and feeling psychologically and physically safe, including being free of and feeling safe from, behaviour of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe.
Sexual assault	A broad term used to describe a range of sexual acts committed against a person without their consent. Sexual assault is a crime of violence where a person uses their power and control to dominate another. Conviction for sexual assault is punishable by law and depending on the category of assault, can carry a gaol term.
Sexual harassment	Can occur between any combinations of individuals and is defined by the Australian Human Rights Commission as 'unwelcome conduct of a sexual nature which makes a person feel offended, humiliated and/or intimidated where that reaction is reasonable in the circumstances.' It can involve physical, visual, verbal or non-verbal contact.
Consensual sexual activity	Any activity of a sexual nature (sexual touching, sexual intercourse, oral sex) that occurs between people over the age of 16 years after mutual sexual consent has been provided by those involved, who have been assessed as having the capacity to consent.
Sexually disinhibited behaviour	Poorly controlled behaviour of a sexual nature, where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way, such as in inappropriate situations, at the wrong time or with the wrong person. This behaviour can arise for a variety of reasons that can include a mental illness or disorder.
Inappropriate context or setting	Due to the vulnerability of mental health consumers this refers to sexual activity between consumers, or consumers and their visitors taking place in an environment or associated with a set of circumstances that is not considered to be suitable, such as: * An acute inpatient setting. * Masturbation in a public area. * Outpatient/non acute setting. * When a staff member suspects a patient has been coerced into engaging in sexual activity.
Child sexual abuse	Child sexual abuse is when a child is forced or persuaded to take part in sexual activities. This may involve physical contact or non-contact activities and can happen online or offline. A child may not always understand that they are being sexually abused. Recognising Child Abuse .
Trauma informed	Trauma-informed care understands and considers the pervasive nature of trauma and promotes environments of healing and recovery rather than practices and services that may inadvertently re-traumatise. ²

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Policy principles

Rockingham Peel Group Mental Health Services will:

- Uphold and promote rights and responsibilities pertaining to sexual safety for consumers, carers/families, and staff.
- Promote and support a sexually safe culture and environment.
- Empower consumers to promote sexual safety.
- Provide and make readily accessible throughout the service clear information on individual rights, acceptable and unacceptable behaviours and processes for reporting or registering a complaint regarding issues of sexual safety to consumers, carers/families.
- Continually improve the services skill and ability in recognition of factors which impact on the sexual safety of consumers.
- Take timely, and appropriate actions to prevent sexual safety incidents.
- Ensure disclosures from consumers about incidents that compromise or breach their sexual safety are taken seriously and addressed appropriately and promptly with regard for the consumer's privacy, dignity, past trauma, cultural background, gender, religion, sexual identity, age, and illness.

Rights and responsibilities in relation to sexual safety

Environment

- All persons accessing our service have the right to feel psychologically, physically safe. This includes feeling safe from behaviours of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid, or unsafe.
- Essential elements of a sexually safe environment:
 - * Trauma informed; and based on the principles of safety, trustworthiness, choice, collaboration, and empowerment and emphasises recovery.
 - * Fair and equitable, available to a diverse range of individuals and free from discrimination.
 - * Compassionate, sensitive and with an open culture that encourages reporting of sexual safety incidents relating to consumers, family/carers/guardians, and staff.
 - * Safe and nonjudgmental for LGBTQIA+ consumers through:
 - Actively promoting the service's acceptance of diverse sexual orientations and gender identities.
 - Asking how consumers identify in relation to gender and the identity terms, names and pronouns that they use and document in their medical records.
 - Using language that is gender neutral.
- Balances personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all consumers, personal support persons, visitors, and staff.

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Statement	Everyone	Consumers
Feel sexually safe whilst in a mental health service.	X	
Expect concerns to be taken seriously.	X	
Be consulted about and provide input to sexual safety standards developed within the service they are involved with.	X	
Express themselves in accordance with their gender identity.	X	
Be heard and believed regarding their experience of feeling unsafe or sexual assault, harassment, and any other breaches of sexual safety.		X
Be treated with respect and dignity.	X	
Receive services free from abuse exploitation, discrimination, coercion, harassment, and neglect.		X
Participate in decisions about their treatment, care, wellbeing and sexual safety.		X
Participate in safety planning.	X	
Receive clear information of rights and responsibilities in relation to sexual safety.	X	
Be treated with compassion and understanding when they disclose incidents that have compromised their sexual safety.		X
Receive a timely response if they disclose an experience of sexual assault or harassment.		X

On entry/admission to the service

- Consumer rights are promoted throughout the service and the Chief Psychiatrist poster highlighting 'Sexual Safety of Consumers' must be displayed in all areas.
- Consumers are provided with an orientation to the inpatient/community setting that includes their right to a safe and therapeutic environment, an explanation that sexual activity on the unit/service is inappropriate and not permitted and that intimate behaviour or sexualised contact with other consumers or staff is not acceptable.
- Consumers must also be informed that taking photographs/videos whilst on any site is not allowed and may breach other consumer's privacy and will not be tolerated
- Staff will recognise the consumers need for privacy and personal space and provide this in line with consideration of all other risk identified through assessment. [RKPG Observations: Safe and Supportive \(Mental Health\) Procedure](#).
- Unless clinically contraindicated all consumers will be asked about their preference for staff gender on a case-by-case basis with consideration to known history and trauma background.
- Staff allocation will be done according to consumer preference, clinical need and staff availability and based on clinical risk assessment.
- All inpatient rooms have an ensuite bathroom which facilitates privacy and dignity for the individual. Inpatients rooms also have a lockable door.

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- For further education clinicians can access the Sexual Assault Resource Centre (SARC) – Sexual Trauma and Responding to Disclosures of Sexual Assault eLearning program. Also refer to:
 - * [King Edward Memorial Hospital - Sexual Assault Resource Centre \(SARC\)](#)
 - * [DoH Complaints Management Policy](#)
 - * [ACSQHC My Health Care Rights Poster A4](#)
 - * [DoH Your rights and responsibilities as a patient](#)

Assessment of sexual risk

- Staff need to be aware of possible past sexual abuse and how this may increase a consumer's vulnerability to further inappropriate sexual activity, exposure, or exploitation by others.
- All consumers' sexual safety issues will be approached in a sensitive and considered manner and will be identified in their assessment and Risk Assessment and Management Plan (RAMP) on admission. Provision for ongoing sexual safety assessment should also occur where indicated throughout the treatment phase.
- Assessment of a consumer's sexual offending risk and sexual vulnerability should include present or past history of exposure to sexual abuse and/or inappropriate sexual activity as part of a developmental history including:
 - * Sexual history, sexual development, sexual and gender orientation, HIV status and any other sexually transmitted diseases.
 - * History of and vulnerability to sexual abuse, assault or harassment.
 - * History of and potential for sexual activity due to illness, emotional state or behaviour.
 - * History of and potential for sexually inappropriate behaviour including sexual disinhibition, sexual harassment or sexual assault.
 - * Any concerns relating to visitors that may contribute to the person's sexual vulnerability or risk of sexually inappropriate behaviour.
 - * History of and potential for violent behaviour, threats or intimidation. The assessment should consider the presence of signs or symptoms that can indicate a previous sexual assault including but not limited to:
 - Disturbed sleep, frequent nightmares, flashbacks to the assault.
 - Embarrassment, shame, eroded self-esteem, poor body image, weight gain used as a protective mechanism.
 - Highly sexualised behaviour, hostility and behavioural problems, self-harming and compulsive behaviours.
 - Relationship difficulties, feelings of isolation, guilt or self-blame, suicidal thoughts and suicide attempts, a range of mental health disorders.
 - Present or past history of exposure to abuse or inappropriate sexual activity.

Treatment and support

- Where there is potential/actual risk identified, visitor interactions will be monitored, any inappropriate behaviour, of a sexual nature will be reported in the consumer's medical record, handover, and clinical reviews.

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- Strategies to empower the consumer and mitigate risks include:
 - * Regulating the use of electronic personal electronic devices where appropriate.
 - * Engaging the consumer in safety planning and protective behaviours in collaboration with staff.
 - * Comprehensive Treatment Support and Discharge Plan (TSDP) documented in the patient's medical record which will include:
 - Patient preferences regarding the gender of clinicians and concerns regarding contact with other consumers.
 - Identified sexual safety issues.
 - Plans to manage sexual disinhibition.
 - Visual observations for patients at risk.
 - How the consumer identifies in relation to gender.
 - Any cultural or multicultural considerations.
- Patients reported to be displaying sexualised behaviours and/or at-risk patients will be highlighted during clinical handover and discussed at team meetings or multidisciplinary (MDT) team review meetings.
- To ensure maximum visibility patients deemed at risk will be allocated bedrooms closest to the nursing station. Two staff, at least one of the same gender are to be present during routine hourly safe checks at night.
- Visual observations of patients will be initiated, maintained and documented in line with the [RKPG Observations: Safe and Supportive \(Mental Health\) Procedure](#) and [RKPG Resource Management for Patients Requiring Increased Observation](#).
- If a consumer exhibits sexually inappropriate behaviour the individual will be offered discreet, non-intrusive and non-custodial measures such as providing feedback about acceptable behaviour and/or providing a temporary location that provides safety for all.
- All patients will be offered the presence of a gender preferred chaperone during any physical examination or procedure in line with the [SMHS Chaperone Policy](#).
- Reporting – DATIX incident review.
- Where there is potential/actual risk identified, visitor interactions will be monitored, any inappropriate behaviour, particularly of a sexual nature will be reported immediately.

Community based services

Clinicians will:

- Identify any sexual safety issues that may arise in the clinics or when visiting the patient in their home and document the management plan in the TSDP.
- Place alerts in the Psychiatric Services Online Information system (PSOLIS) when appropriate.

Sexual safety incidents

- Sexual assault can take many forms including:
 - * Being forced to masturbate or watch another masturbate.
 - * Being forced, coerced, or bribed to view pornographic images.

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- * Being forced to give or receive oral sex.
- * Being forced to perform sexual acts on themselves or others; and
- * Sexual penetration of a person by penis, object, or other part of a body into the vagina, anus or mouth.
- Where a consumer has indicated a sexual assault has occurred it is paramount to minimise the impact on their mental health by responding in a consistent manner to disclosure of an allegation of sexual assault.
- The primary steps in response by clinicians should be to:
 - * Assess the patient's immediate safety and wellbeing and need for medical treatment.
 - * Ensure the patients right to privacy and confidentiality are respected
 - * Provide an appropriate chaperone.
 - * Provide appropriate support/treatment and or referral to services e.g. SARC, Police.
 - * Document and report all sexual safety breaches including reports of alleged sexual assault, harassment, sexual activity, sexually disinhibited behaviour.
 - * Place an alert on PSOLIS for patients with known/reported offences.
 - * Identify and document priority actions to increase sexual safety.
 - * [DoH Responding to an Allegation of Sexual Assault disclosed within a Public Mental Health Service.](#)
 - * Inform their immediate line manager and Consultant Psychiatrist of all sexual breaches.
 - * Follow the guidelines outlined in:
 - [DoH WA Notifiable and Reportable Conduct MP 0125/19](#)
 - [DoH WA Complaint Management Policy MP0130/20](#)
 - [Policy for Mandatory reporting of notifiable incidents to the Chief Psychiatrist](#)
- Provide staff with the opportunity to debrief and /or access to the Employee Assistance Program.

Staff safety

- Mental Health staff have the right to feel safe while in the mental health inpatient environment or in the course of their work in the community.
- Risk to staff will be assessed by considering:
 - * PSOLIS alerts.
 - * Any previous history of boundary violations with previous staff.
 - * History of false allegations or complaints of a sexual nature.
- Staff members should take care not to place themselves in vulnerable settings. whereby the potential for allegations may arise e.g. isolated in a patient's bedroom.
- Community staff will comply with the [RKPG Community/Home Visiting Policy](#).
- Staff in all settings will be provided with optimal opportunity to attend training on managing sexual safety, trauma informed care and reporting of sexual safety incidents.

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Education

- For further education clinicians can access a SARC – Sexual Trauma and Responding to Disclosures of Sexual Assault eLearning program.

Compliance monitoring

The Mental Health Service will conduct an annual sexual safety audit to assess sexual safety within the inpatient setting. The audit report will be tabled at the Mental Health Safety Quality Sub-Committee.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>
2. Phoenix Australia, National Centre of Excellence in Posttraumatic Mental Health. Trauma-informed Care. 2024. Available from <https://www.phoenixaustralia.org/consultation-services/trauma-informed-care/>

Links

[ACSQHC National Standards in Mental Health Services](#)
[Australian Human Rights Commission](#)
[Chief Psychiatrists of Western Australia Standards for Clinical Care](#)
[Criminal Code Act 1913](#)
[Department of Health \(DoH\) Code of Conduct Policy MP0124/19](#)
[DoH Discipline Policy MP0127/20](#)
[DoH Notifiable and Reportable Conduct Policy MP 0125/19](#)
[DoH Complaints Management Policy MP0130/20](#)
[DoH Clinical Incident Management Policy MP0122/19](#)
[Policy for Mandatory reporting of notifiable incidents to the Chief Psychiatrist](#)
[DoH State-wide Standardised Clinical Documentation MP0155/21](#)
[Mental Health Act 2014](#)
[NSW Ministry of Health Quick Guide for Busy Clinicians - Integrated Trauma informed Care Framework](#)
[RKPG Community/Home Visiting Policy](#)
[RKPG Observations: Safe and Supportive \(Mental Health\) Procedure](#)
[RKPG Resource Management for Patients Requiring Increased Observation \(Acute\) Policy and Procedure](#)
[SMHS Chaperone Policy](#)
[SMHS Rights of patient in Mental Health Policy](#)

National Standards



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